

Bariatric follow up

General Practice Guidelines

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Introduction

This guidance covers the Devon bariatric surgery patient pathway and associated post-surgery follow up care.

It details the follow up requirements for patients who are following a pathway for NHS funded surgery and for those who are planning to have, or have had, self-funded bariatric surgery.

It has been produced to guide general practice with the delivery and good practice of bariatric follow up care and is supported by several other resources available inclusive of NHS Devon Local Enhanced [Service Specification](#) for Bariatric Follow Up and a [Patient Information Leaflet](#) for people considering privately funding bariatric surgery.

NHS funded bariatric surgery

Patients have a legal right to choose their NHS provider for bariatric surgery if the chosen provider is clinically appropriate and has an NHS contract for that service.

Many patients registered with a Devon GP practice, choose to have their bariatric surgery at University Hospitals Plymouth (UHP) or Musgrove Park Hospital Taunton (MPH). The pathway from referral to surgery takes approximately 12 months unless patients meeting specific clinical criteria are identified as appropriate for a straight to bariatric surgery assessment with the specialist bariatric service.

The main referral route to bariatric surgery is via Tier 3 Specialist Weight Management Services who will undertake an initial assessment of suitability for surgery.

To be eligible for NHS funded bariatric surgery patients are required to have been through a Tier 3 Specialist Weight Management Programme, have met the objectives of the programme and for bariatric surgery to have been deemed the most appropriate treatment option by the specialist weight management multidisciplinary team.

Once a referral has been received by the specialist bariatric service there are several stages in the pathway that patients go through prior to surgery inclusive of education workshops, nutritional blood screening and a liver reduction diet.

Once surgery has been undertaken, specialist services shall inform general practice of the surgery undertaken, post operative care, prescribe medication and follow up appointments.

Requirements for post-surgery care (up to two years post-surgery)

NICE NG246 Obesity Clinical Guideline (January 2025) states that patients should be offered a follow up package of care for a minimum of two years with the specialist bariatric service who have undertaken the surgery.

Within Devon patients are provided with this follow up from the specialist services who have provided the surgery for the first two years post-surgery.

Following this the specialist bariatric services will discharge patients to general practice for their ongoing, lifelong follow up care. As part of this discharge the specialist services will provide a discharge letter for the patients GP outlining any patient goals, required interventions and follow up requirements.

Requirements for lifelong follow up care (from two years post-surgery)

Patients who undergo bariatric surgery should have lifelong follow up care inclusive of annual monitoring of nutritional status and appropriate supplementation. If patients receive inadequate follow up there is a potentially serious risk of malnutrition and the consequences of this.

Patients are informed of the need for this lifelong care and supplementation at multiple points during their pathway (assessment for readiness for bariatric surgery, at referral to the surgical team, during education in preparation for bariatric surgery and at consent for surgery).

Lifelong follow up care post two-years surgery is not mandated for practices as part of their core contract. For practices that have signed up to deliver the bariatric follow up LES, this is to be provided as part of a collaborative care model between general practice and specialist bariatric services with shared requirements as detailed in the NHS Devon Local Enhanced [Service Specification](#) for Bariatric Follow Up.

Privately funded bariatric surgery

Provision of standalone NHS post bariatric surgery follow up care for patients who have self-funded bariatric surgery is not routinely commissioned.

Requirements for post-surgery care (up to two years post-surgery)

For patients who choose to self-fund bariatric surgery either in the UK or overseas, the requirements for follow up care for the first two years post-surgery lies with the private provider who has performed the surgery.

For patients who are considering self-funding bariatric surgery, they should be advised that they will not be eligible for NHS follow up for the first two years post-surgery if they have not been through an NHS Specialist Weight Management Programme and will therefore be required to arrange independent follow up care.

An [information leaflet](#) has been developed to support patients who are considering self-funding bariatric surgery.

Information should be provided to patients regarding the recommended post bariatric surgery supplementation in accordance with the British Obesity and Metabolic Surgery Society [BOMSS](#) guidance. Patients need to be advised that they are required to purchase the recommended supplements independently.

Emergency care: the only exception is when a patient requires emergency care, or the complication is life threatening. In these cases, for patients presenting in general practice they should be referred to the appropriate emergency team via existing pathways.

Requirements for lifelong follow up care (from two years post-surgery)

Lifelong follow up in general practice applies for practices who have signed up to deliver the NHS Devon bariatric follow up LES, for patients post-two years private bariatric surgery in line with the NHS Devon Local Enhanced [Service Specification](#) for Bariatric Follow Up.

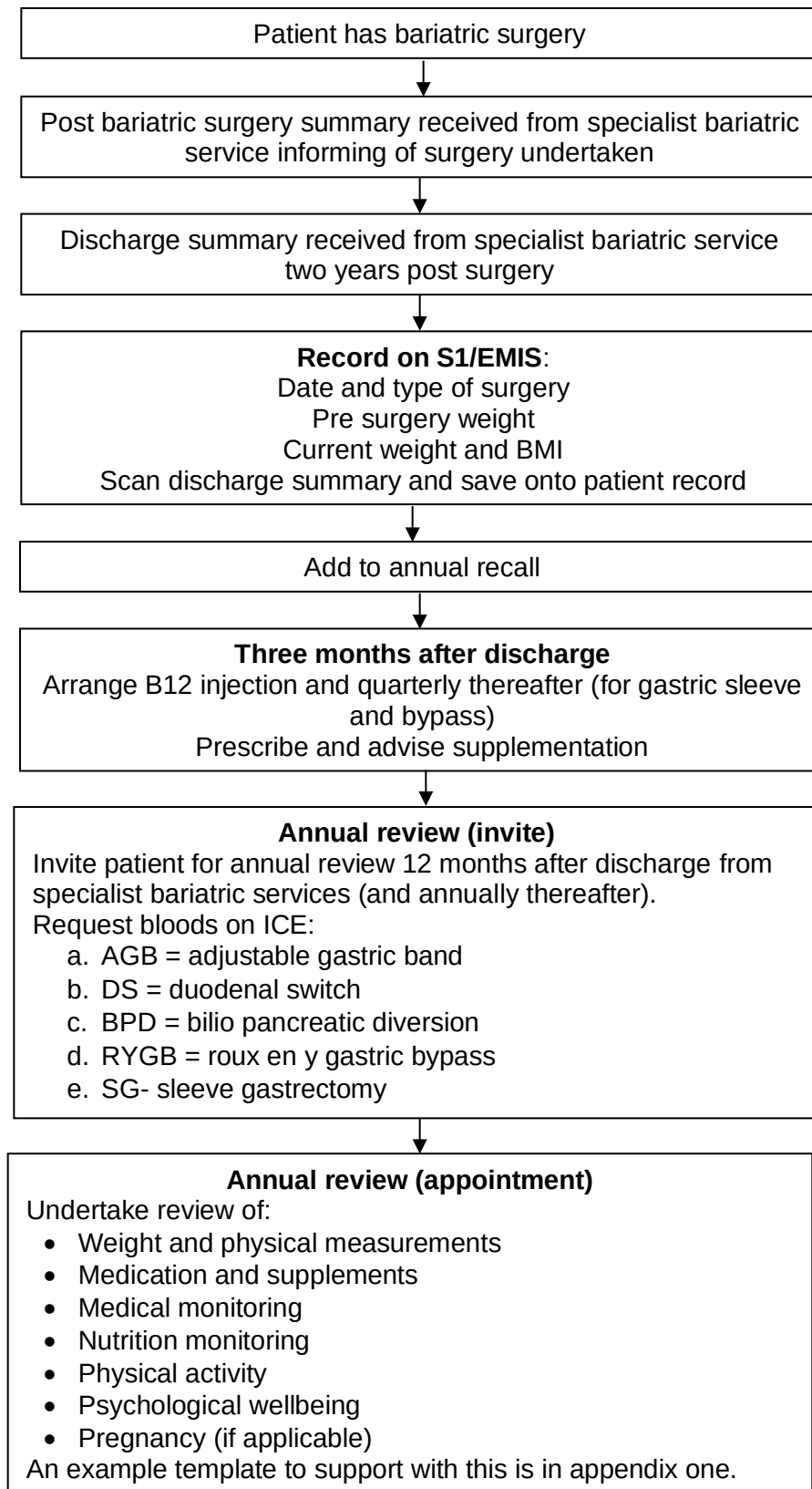
Follow up process (from two years post-surgery)

The requirements for arranging annual reviews and associated blood tests lies with patients; this is communicated to patients at various stages of their Tier 3 and Bariatric Surgery pathways.

Practices may choose to send annual recall letters advising patients to book an appointment for the recommended annual post-bariatric surgery blood tests, along with a GP annual review appointment once the blood results are available.

The algorithm below provides an overview of the follow up process.

Lifelong follow up process in general practice



Complications

For patients presenting with complications where specialist support is required throughout their follow up care, and the information is not covered in the [BOMSS](#) guidance, there is a fast-track referral pathway back to the specialist bariatric services to escalate care.

The contact details to refer patients back into the surgical teams are as follows:

Plymouth

Email: plh-tr.derrifordbariatrics@nhs.net

Taunton

Email: bariatricsurgerydietitians@somersetft.nhs.uk

Additional information

Supplementation

Supplementation is required life-long and needs to be taken by all patients to prevent any vitamin or mineral deficiencies developing. This should be provided in line with [North and East](#) Devon / [South and West](#) Devon local formulary and [BOMMS](#) guidance.

Contraception and pregnancy

Oral contraceptive tablets may not be fully absorbed so discuss contraceptive options within the annual review as required.

There is an increased risk of adverse outcomes in pregnancy for patients who have had bariatric surgery. These risks are increased during the rapid weight loss phase and patients are advised to avoid getting pregnant within the first 18 months after surgery.

For patients planning pregnancy or who become pregnant refer them to a bariatric specialist as they may need advice on changing vitamin supplements to avoid those containing vitamin A, and they may need high dose folic acid 5mg.

Further information is covered in [BOMSS](#) guidance.

Procedures to remove excess skin

Apronectomy procedures or procedures to remove excess skin at other anatomical sites are not routinely commissioned for patients post bariatric surgery.

Specialist weight management services do not support referral to the Individual Funding Requests Panel unless patients are experiencing recurrent sepsis (requiring at least one admission) within the skin folds.

Patients should also be made aware that they should expect excess skin after their bariatric surgery and that removal of this is not routinely funded on the NHS, unless they are experiencing significant, recurrent medical issues.

The NHS Devon cosmetic policy can be found [here](#).

Additional resources

National guidelines

Further guidelines for GPs for post bariatric surgery care are available via [BOMSS](#). The NICE NG246 Obesity Clinical Guideline (January 2025) is available via [NICE](#).

Local guidelines

NHS Devon Commissioning Policy for Bariatric Surgery is available on the [One Devon](#) website

Additional local resources

Bariatric nutrition

A video produced by LWSW Tier 3 specialist weight management services has been produced to support with bariatric nutrition and is available on [You Tube](#). This is not publicised in the public domain.

Dumping syndrome and reactive hypoglycaemia

A patient information [leaflet](#) has been produced by UHP to provide information on dumping syndrome and reactive hypoglycaemia.

My Health Devon

Patient [information](#) on weight management support inclusive of local services available.

Additional local services

Talkworks (excluding Plymouth)

Devon NHS Talking Therapies including support for binge eating

More information: www.talkworks.dpt.nhs.uk

Telephone: 0300 555 3344

Plymouth Talking Therapies (Plymouth only)

More information: www.livewellsouthwest.co.uk/plymouth-options

Telephone: 01752 435 419

West Devon and Plymouth Eating Disorder Service

Help and support for people struggling with the impacts of an eating disorder.

More information: www.eatingdisorderserviceplymouth.co.uk

Telephone: 01752 228027

Appendix one

The information below is the suggested information to be recorded on practice systems as part of annual reviews in line with the NHS Devon Local Enhanced Scheme for Bariatric Follow Up.

Weight and physical measurements

- Current weight in kilograms

Medication and supplements

- Is the patient taking any of the following:
 - a. Complete A to Z multivitamin and mineral
 - b. Calcium and vitamin D supplement (not needed for gastric band)
 - c. Iron tablet (not needed for gastric band)
- Date of last B12 injection
Reiterate requirement for quarterly B12 injections (not needed for gastric band).

Medical monitoring

- Does the patient have any of the following gastrointestinal symptoms:
 - Reflux
 - Nausea
 - Vomiting
 - Dumping syndrome
 - Abdominal pain
- Does the patient have any of the following pre-existing conditions:
 - Type 2 diabetes
 - Obstructive sleep apnoea
 - Atherosclerotic Cardiovascular Disease (ASCVD)
 - Hypertension
 - Dyslipidaemia

If yes to the above, date of last review of pre-existing conditions.

Nutrition monitoring

- Does the patient have any of the following nutritional concerns:
 - Poor appetite
 - Food intolerances
 - Chewing difficulties

Physical activity review

- Current activity level:
 - Sedentary
 - Moderate

- Active
- Participation in structured exercise:
 - Yes
 - No

Psychological wellbeing

- How often has the patient been bothered by the following over the past 2 weeks:
 - Feeling nervous, anxious, or on edge
 - Not at all
 - Several days
 - More than half the days
 - Nearly every day
 - Not being able to stop or control worrying
 - Not at all
 - Several days
 - More than half the days
 - Nearly every day
 - Little interest or pleasure in doing things?
 - Not at all
 - Several days
 - More than half the days
 - Nearly every day
 - Feeling down, depressed, or hopeless?
 - Not at all
 - Several days
 - More than half the days
 - Nearly every day

Pregnancy (if applicable)

- Contraception use
 - Yes
 - No
- Pregnancy plans
 - Yes
 - No
- Menstrual irregularities
 - Yes
 - No

Other

- Does the patient have any other concerns regarding their bariatric surgery?

Follow up and action plan

- Further investigations required
- Medication adjustments
- Onward referrals needed:
 - Dietician
 - Psychological services
 - Endocrinologist
 - Support group
- Date of next B12 injection (not required for gastric band)
- Date of next annual review